

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: A Phase 3, Randomized, Double-blind, Placebo-controlled Study to Evaluate the Efficacy and Safety of Deucravacitinib in Adults With Active Sjögren's Syndrome (POETKY SJS-1) **Short Title/Acronym:** POETKY SJS-1 **Protocol Number:** IM011-1069 **NCT Number:** NCT05946941 **Posting ID:** 233053399496912169 **Sponsor/Company Name:** Bristol-Myers Squibb **Trial Status:** Recruiting **Investigational Product:** Deucravacitinib **Trade Name:** Sotyktu **ATC Code:** L04AA56 **Phase of Development:** Phase 3 **Medical Monitor:** GSM-CT Representative (Contact: mg-gsm-ct@bms.com)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of deucravacitinib versus placebo by measuring improvements in the EULAR Sjögren's Syndrome Disease Activity Index (ESSDAI) score at Week 52. **Secondary Objectives:** To assess the safety and tolerability of two doses of deucravacitinib; and to evaluate improvements in patient-reported symptoms such as fatigue, dryness, and joint/muscle pain.

2.2 Background & Rationale Sjögren's Syndrome is a complex, chronic inflammatory and autoimmune disease in which the salivary and lacrimal glands undergo progressive destruction by lymphocytes and plasma cells resulting in decreased production of saliva and tears (sicca symptoms), for which there is currently no approved targeted treatment. Deucravacitinib is a first-in-class, oral, selective, allosteric tyrosine kinase 2 (TYK2) inhibitor. By specifically targeting cytokine pathways involving IL-12, IL-23, and type I interferons, deucravacitinib has the potential to precisely control symptoms and reduce inflammation and glandular damage in SJS patients with a favorable safety profile compared to broader immunosuppressants.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized (1:1:1 allocation to deucravacitinib 3 mg twice daily, deucravacitinib 6 mg twice daily, or matching placebo)

3.2 Planned Enrollment & Duration Target **SN\$:** 756 participants **Number of Sites:** Approximately 215 global locations **Estimated**

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male or female ≥ 18 years of age. 2. Satisfy the 2016 American College of Rheumatology/European League Against Rheumatism (ACR/EULAR) criteria for the classification of SjS at screening (score ≥ 4) with a disease duration of at least 16 weeks and ≤ 10 years before screening. 3. Have moderate to severe SjS with an ESSDAI score ≥ 5 at screening (across 8 organ domains). 4. Positive anti-Sjögren's syndrome-associated antigen A (anti-Ro/SSA) antibody at screening. 5. Stimulated whole salivary flow (SWSF) ≥ 0.05 mL/minute (or unstimulated ≥ 0.01 mL/minute).

4.2 Exclusion Criteria 1. Autoimmune disease other than SjS (e.g., rheumatoid arthritis, systemic lupus erythematosus, systemic sclerosis). 2. Active fibromyalgia with pain symptoms or signs that would interfere with joint assessment or requiring adjustment in medication within 3 months before screening to control symptoms (participants with well-controlled fibromyalgia on stable treatment may be considered). 3. Medical condition associated with sicca syndrome. 4. Previous exposure to tyrosine kinase 2 (TYK2) inhibitors such as deucravacitinib or related compounds.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Deucravacitinib 3 mg twice daily or 6 mg twice daily. **Route of Administration:** Oral (pill) **Duration of Treatment:** 52 weeks for the double-blind treatment period, followed by an optional Long-Term Extension (LTE) period of up to 2 additional years (up to 156 weeks).

5.2 Concomitant & Prohibited Therapy Permitted: Stable baseline treatments for SjS as specified per protocol (e.g., well-controlled fibromyalgia treatments). **Prohibited:** Prior use of TYK2 inhibitors.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Up to 28 days (max 56)	Informed Consent, baseline ESSDAI assessment, Anti-Ro/SSA testing, salivary flow tests
Baseline	Day 0	Randomization, First Dose
Treatment Period	Up to Week 52	Efficacy (ESSDAI & ESSPRI) & Safety assessments, evaluation of fatigue, dryness, and pain
Long-Term Ext.	Weeks 52 to 156	Optional blinded

continuation (placebo patients switch to active deucravacitinib)
| | **End of Study** | 4 weeks post-treatment| Safety Follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Change from baseline in the European League Against Rheumatism Sjögren's Syndrome Disease Activity Index (ESSDAI) score evaluated at Week 52. **Measurement Method:** Clinical assessment measuring disease activity across physiological/organ domains.

7.2 Secondary & Exploratory Endpoints - Change from baseline in European League Against Rheumatism Sjögren's Syndrome Patient Reported Index (ESSPRI) Score at Week 52. - Number of participants with decrease in ESSPRI ≥ 1 or 15% from baseline at Week 52. - Number of participants with decrease in ESSDAI ≥ 3 points from baseline at Week 52. - General safety and adverse event (AE) evaluations across treatment periods.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection evaluating safety profiles and monitoring for side effects across the 52-week double-blind period and the LTE period. **Serious Adverse Events (SAEs):** Evaluated strictly throughout treatment and up to the 4-week safety follow-up. **Data Monitoring Committee (DMC):** Overseen by the sponsor (Bristol-Myers Squibb) to ensure safety signals are identified and acted upon.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy; Safety Set for all participants receiving ≥ 1 dose of the study drug. **Sample Size Justification:** Approximately 756 participants powered to detect a statistically significant change in ESSDAI score reduction and ESSPRI improvements comparing active doses against placebo. **Handling of Missing Data:** Standard imputation methods applicable to chronic autoimmune phase 3 clinical trials.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring to ensure protocol adherence, objective ESSDAI scoring, and patient safety. **Audit Trail:** Secure management of eCRFs and symptom tracking forms across all global sites.

11. Study Identifiers & Governance

Primary ID: IM011-1069 **Registry Number:** NCT05946941 **Posting ID:** 233053399496912169 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** POETYK Sjs-1 **Official Regulatory Title:** A Phase 3, Randomized, Double-blind, Placebo-controlled Study to Evaluate the Efficacy and Safety of Deucravacitinib in Adults With Active Sjögren's Syndrome

12. Clinical Framework (Sjögren's Specific)

Primary Condition: Active Sjögren's Syndrome (Sjs) **Preferred UMLS Name:** Sjögren's Syndrome **MeSH Code:** D012859 **SNOMED ID:** 83901003 **Diagnosis Threshold:** ACR/EULAR 2016 criteria score ≥ 4 ; ESSDAI ≥ 5 ; Positive Anti-Ro/SSA. **Medical Methodology:** Selective TYK2 Inhibition targeting IL-12, IL-23, and type I interferons. **Optimization Target:** Reduction in ESSDAI disease activity score and mitigation of hallmark symptoms (measured by ESSPRI).

13. Study Procedures & Methodology

Management Pathway: Targeted synthetic disease-modifying management for autoimmune condition. **Education Protocol (HCP):** Training on standardizing the calculation of the 8-domain ESSDAI and correctly measuring SWSF. **Education Protocol (Patient):** Informed consent detailing the 1-in-3 randomization chance (placebo vs. active drug) and instructions on LTE continuation. **Quality Audit Mechanism:** Periodic review of ESSDAI calculations, salivary test logs, and lab results monitoring the inflammatory markers.

14. Observation & Follow-Up Schedule

Total Duration: Up to 60 weeks for main study (4 weeks screening + 52 weeks treatment + 4 weeks follow-up); up to 160 weeks if continuing on the LTE. **Onsite Visit Cadence:** Regular clinical evaluation points (including Week 24 and Week 52). **Remote Monitoring Frequency:** Consistent remote checks per sponsor plan. **Checklist Review Interval:** Regular recording of symptomatic changes in dryness, fatigue, and pain.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Insert Name]	____	[DD-MMM-YYYY]				

| **Clinical Operations Lead** | **[Insert Name]** | ____ | **[DD-MMM-YYYY]**
| | **Lead Statistician** | **[Insert Name]** | _____ | **[DD-MMM-YYYY]** |